

Psoriatic Arthritis vs Rheumatoid Arthritis: Key Differences

Practical NHS-backed advice on psoriatic arthritis vs rheumatoid arthritis differences.

By **Living With Arthritis UK** · Reviewed by **Louis Maxwell** (HCPC Registered Physiotherapist) · 2026-07-24 · livingwitharthritis.org.uk

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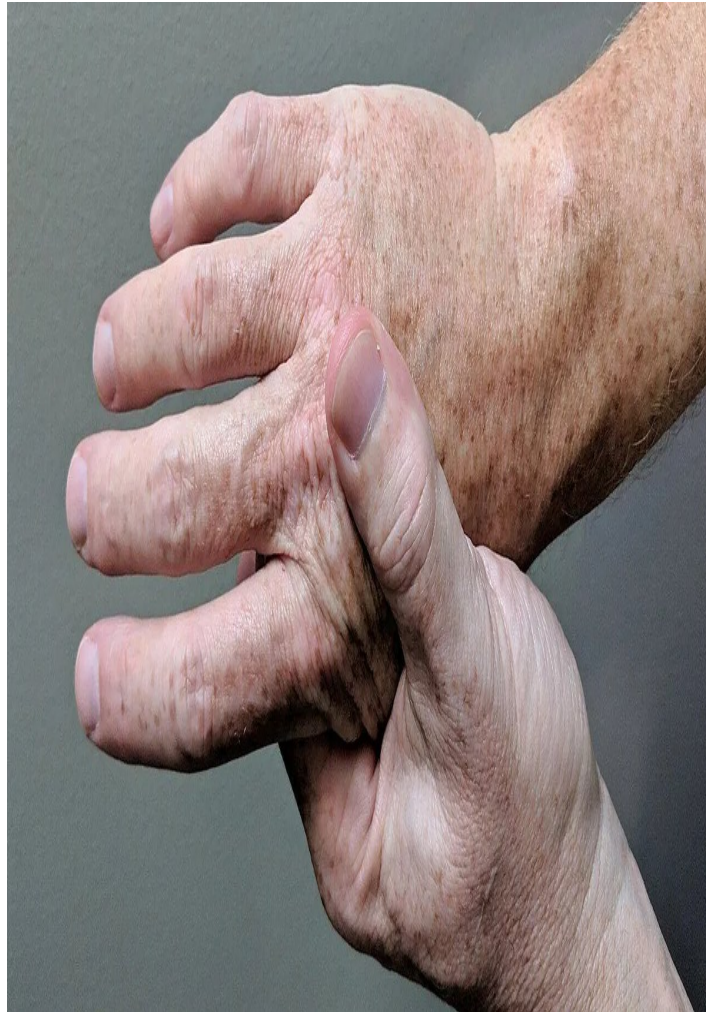
Quick Answer: Psoriatic arthritis (PsA) is linked to psoriasis and often affects the spine and fingers asymmetrically. Rheumatoid arthritis (RA) is an autoimmune condition causing symmetric joint inflammation, usually diagnosed via a positive RF or anti-CCP blood test. Both require specialist treatment but use partly different medications.

Both conditions cause joint pain, stiffness, and swelling, but differ in cause, pattern, and treatment. Knowing which you have is essential for the correct treatment pathway.

What Is Psoriatic Arthritis?

PsA affects approximately 30% of people with psoriasis. It causes inflammation in joints, tendons (enthesitis), and fingers/toes (dactylitis — "sausage digits"). It often affects the spine asymmetrically and is usually seronegative (blood tests for RF are negative).

What Is Rheumatoid Arthritis?



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RA is an autoimmune disease where the immune system attacks the joint lining. It causes symmetric joint inflammation (both wrists, both knuckles), pronounced morning stiffness, and is associated with fatigue and anaemia. Approximately 70–80% of RA patients test positive for rheumatoid factor or anti-CCP.

Key Differences at a Glance

- Skin: PsA linked to psoriasis; RA has no skin connection
 - Pattern: PsA often asymmetric; RA typically symmetric
 - Spine: PsA commonly affects spine; RA rarely does
 - Nails: PsA causes nail pitting; RA does not typically affect nails
 - Blood tests: RA often RF/anti-CCP positive; PsA usually seronegative
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Treatment Differences

Both use NSAIDs and methotrexate first-line. However, PsA may additionally use IL-17 inhibitors (secukinumab) and IL-23 inhibitors not used in RA. Accurate diagnosis therefore determines which biologics you can access on the NHS.

Q&A;



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Question: Can you have both PsA and RA at the same time? **Short answer:** It is extremely rare. A rheumatologist will use clinical features, imaging, and blood tests to distinguish between them accurately.

Question: Does PsA always come with visible psoriasis? **Short answer:** No — in about 15% of cases, joint symptoms appear before skin symptoms. Family history of psoriasis is still an important diagnostic clue.

Question: Which condition is more severe? **Short answer:** Both range from mild to severe. Modern treatments for both are highly effective when started early. Early rheumatology referral is the single most important factor for good outcomes in both conditions.

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